

Moxipen[®]

Capsule 250mg / Powder For Oral Suspension

Composition

Moxipen Capsule 250mg

Each capsule contains Amoxycillin Trihydrate equivalent to Amoxycillin Anhydrous 250 mg.

Moxipen Oral Suspension

Each 5 mL reconstituted suspension contains Amoxycillin Trihydrate equivalent to Amoxycillin Anhydrous 125 mg. Sodium Benzoate 0.1% w/v as preservative.

Pharmacodynamics

Amoxycillin is bactericidal and has a broad-spectrum activity which is effective against both Gram-positive and Gram-negative organisms, including *Haemolytic streptococci*, *Diplococcus pneumoniae*, non-penicillinase-producing *Staphylococci*, *Bacillus anthracis*, *Clostridia* spp, *Corynebacterium xerose*, and most strains of *Enterococci*, *Haemophilus influenza*, *Neisseria gonorrhoea*, *Neisseria meningitidis*, *Proteus mirabilis*, many strains of *Salmonella*, and *Escherichia coli*. It is less effective than ampicillin against *Shigella* spp., and more effective against *Streptococci* and *Salmonella* spp.

Pharmacokinetics

Amoxycillin is resistant to inactivation by gastric acid. It is more rapidly and more completely absorbed than ampicillin when given orally. The presence of food in the stomach does not appear to reduce the total amount absorbed. About 20% is bound to plasma proteins and plasma half-lives of 1 to 1.5 hours have been reported. Amoxycillin is widely distributed at varying concentrations in body tissues and fluids. It crosses the placenta; small amounts are distributed into breast milk. About 60% of an oral dose of amoxycillin is excreted unchanged in the urine in 6 hours by glomerular filtration and tubular secretion. High concentrations have been reported in bile; some may be excreted in the faeces.

Indications

Moxipen is indicated in the treatment of:

Ear and Throat Infections: Caused by streptococci, pneumococci, non-penicillinase-producing staphylococci and *Haemophilus influenza*;

Genitourinary Tract Infections: Caused by *Escherichia coli*, *Proteus mirabilis* and *Streptococcus faecalis*;

Skin and Soft Tissue Infections: Caused by streptococci, non-penicillinase-producing staphylococci and *E. coli*;

Acute Uncomplicated Nongenital and Urethral Gonorrhoea: In males and females caused by *N. gonorrhoeae*.

Dosage and Administration

For oral administration only.

Moxipen is stable in the presence of gastric acid and may be given before or after food.

Adults and children 20 kg of body weight and above:
250 - 500 mg every 8 hours.

Infants up to 6 kg of body weight:

25 - 50 mg every 8 hours.

Infants 6 - 8 kg of body weight:

50 - 100 mg every 8 hours.

Infants and children 8 - 20 kg of body weight:

6.7 - 13.3 mg/kg of body weight.

Gonorrhoea: 3 g as a single dose with 1 g of probenecid.

Treatment should be continued for a minimum of 48 to 72 hours beyond the time that the patient becomes asymptomatic or evidence of bacterial eradication has been obtained.

To reconstitute: Shake to loosen powder. Add water and shake vigorously to disperse powder. Add water to the 60 ml mark and shake well. Use within 5 days after reconstitution.

Contraindications

Moxipen is contraindicated in patients allergic to penicillins and in patients with infectious mononucleosis.

Precautions

Patients with impaired renal function may require smaller doses. Avoid prolonged use as this may lead to overgrowth of non-susceptible organisms.

As with any other potent drug, monitoring of renal, hepatic and hematopoietic functions should be carried out during prolonged therapy. Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Moxipen, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenemes or other beta-lactam agents. If an allergic reaction occurs, Moxipen must be discontinued immediately and appropriate alternative therapy instituted.

The safety of **Moxipen** for use during pregnancy has not been established.

Incompatibility and Drug Interactions

None known.

Side Effects

As with other penicillins, the following adverse reactions may occur:

Hypersensitivity Reactions: Erythematous maculopapular rashes and urticaria which can be controlled with antihistamine and if necessary, systemic corticosteroids. If the hypersensitivity reactions occur, treatment should be discontinued.

Gastrointestinal: Nausea, vomiting and diarrhoea.

Liver: A moderate increase in serum concentration of AST (SGOT) has been noted, but the significance of this finding is unknown.

Blood Dyscrasia: Eosinophilia, haemolytic anaemia, anaemia, leukopenia, neutropenia, agranulocytosis, thrombocytopenia, thrombocytopenia purpura and acute interstitial have been reported during therapy with penicillins. These reactions are usually reversible on discontinuation of therapy and are believed to be hypersensitivity phenomena.

Skin and Subcutaneous Tissue Disorders:

Frequency 'very rare': Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS).

Symptoms and Treatment of Overdosage

Moxipen is virtually non-toxic to man. Excessive doses may have irritative effects on the gastrointestinal tract, causing nausea with or without vomiting and mild to severe diarrhoea. There is no specific antidote. Symptomatic treatment should be given.

Shelf-life

Moxipen Capsule 250mg: The expiry date is indicated on the packaging.

Moxipen Oral Suspension: The expiry date is indicated on the packaging.

Presentation

Moxipen Capsule 250mg: Maroon and yellow capsules with 'XSP' printed. Available in packs of 1000's.

Moxipen Oral Suspension:

Before reconstitution: white, free flowing powder with tutti-frutti odor.

After reconstitution: yellow, tutti-frutti flavour suspension.

Storage

Moxipen Capsule 250mg: Store below 30°C.

Moxipen Oral Suspension:

Before and after reconstitution: Store in a cool, dry place below 25°C

KEEP OUT OF REACH OF CHILDREN

JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

Revision Date : 25-Mar-2020



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